جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شيماء يحي محمد علي عطيه
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** 89 ش ابراهيم
رقم البطاقة : 29003140101701 **رقم التليفون :** 01110719209 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 14-03-1990 **المهنة :** حاصل علي بكالوريوس

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شيماء يحي محمد علي عطيه
National ID: 29003140101701

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

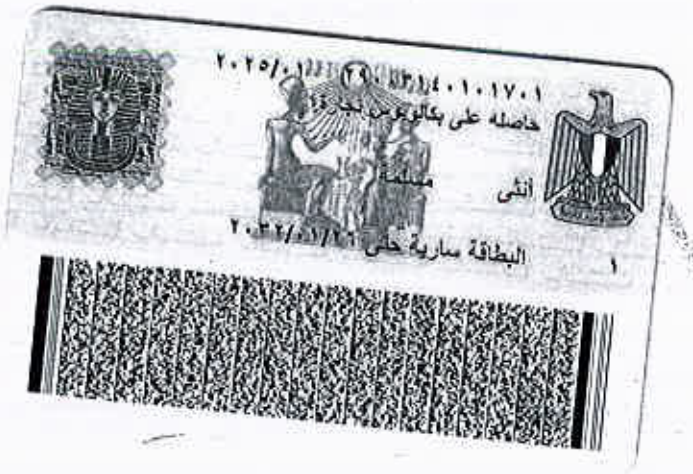
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
شيماء
يحيى محمد على عطيه
٢٨ش الشهيد هاني عبدالوهاب
اول السلام - القاهره
٢٩٠٠٣١٤٠١٠١٧٠١
KZ0419241



Name: شيماء يحي محمد علي **MR. :** 123	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

HER2 enriched under FUP MRI brain 17-2-2026 >>> FREE BSM , CT CAP >>> R O/E >> FREE

4/2026 ct cap : pulmmonary nodules 5mm for follow up

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **Reham Ahmed Fathy Ibrahim**

يعتمد ,

Patient Name	:	Request No.	: 138317
Patient No	: 29003140101701	Request Date	: 23/11/2021
Path No	: SR-665/21	Verification Date	: 25/11/2021
Sex	: Female	Financial Category	:
Age	: 31 Y	Department	: Out Patient
Referreing Doc	:		

Pathology Report

Clinical Data.
Lt breast cancer
Nature of the specimen:
Slide revision
Gross Examination:
one blocks (7898/21), & three stained slides

Microscopic examination:
Histological type: IDC
Grade II
DCIS component:focal
Angioinvasion: no

Diagnosis:
Lt breast cancer, slide revision:
- Invasive duct carcinoma GIII
- Biomarkers:

o ER: negative nuclear reaction in tumor cells (Allred score 0/8)
o PR: negative nuclear reaction in tumor cells (Allred score 0/8)
o Her2: Strong complete membrane reaction in > 10% of tumor cells (score 3+)

Prof.Gamal Abdelaty
Dr.Rana Fathy

Patient : Shimaa Yehia Mohamed

Date: 4/6/2024

^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

- Compared to the previous bone scan report dated 5/1/2022 , the current scan revealed :
- Unfortunately there is no available bone scan film for comparison
- Newly seen Warm radiotracer uptake is seen opposite the left sterno-clavicular region of un-ascertain nature for radiological , clinical correlation
- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- Other wise , no definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are normally seen.

Conclusion:

Follow up bone scan , shows , newly seen warm radiotracer uptake is seen opposite the left sterno-clavicular joint for further clinical –radiological correlation .

Otherwise no definite hot spot that could account for osseous metastatic deposits .

Thanks for referral

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR Mohamed Emam

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
المراكز الطبية المتخصصة
المركز أورام دار السلام
إدارة الأشعة

Patient Name: Shaimaa Yahia Mohamed Ali	Date : 14/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 34 Year(s)
Medical ID: 5(10/2021	Pat. ID: 22452

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast cystic masses
- Right 12:00 well-circumscribed isoechoic oval shaped solid lesion measuring 9.3x6 mm
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right stationary benign looking solid lesion (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada
14/12/2024 10:52 AM

YAHYA, SHIMAA Patient ID: 10110224010474 DOB: 14-Mar-1990 Date: 10-Oct-2024
Patient Lastname: Yahya
Patient Firstname: Shimaa
HIS Patient ID: 10110224010474
RIS Patient ID: 360354
Date of Birth: 14/03/1990

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

REPORT TEXT

99mTc MDP Bone Scan Report (10/10/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc
Revealed:

" No definite evidence of active osseous deposits seen all over the surveyed skeleton.
" Degenerative changes over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion: -
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Alaa Elsaeed
Mohamed Abdelhamid, MSc

REPORT TEXT END

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / شيماء وحي محمد على عطوه	Request No.	: 13000185
Patient No.	: 29003140101701	Request Date	: 27/04/2022
Path No.	: 1162/2022	Verification Date	: 16/05/2022
Sex	: Female	Financial Category	: اورام دار السلام - تكلفة الدولة
Age	: 32 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast mass, post NACT.

Nature of the specimen: Left simple mastectomy specimen.

Gross Examination:
Two specimens received designated as:
1- Sentinel lymph nodes: Fibrofatty tissue piece; 1.5x1 cm. entangling 6 nodules. measured up to 1 cm in diameter
2- Left simple mastectomy specimen measured 22x18x 3cm with unremarkable skin covering measured 18x13 cm. cut section revealed UOQ an ill defined hemorrhagic and grayish white rubbery tumoir bed site, measured 5x4x4 cm, located 2cm from skin and 2 cm from deep margin
• Frozen section: Free SLN 0/6.

Microscopic examination:
• No remanats of tumour tissue.
• Free Nipple and areola.
•Lymph node involvement: sentinel lymph node 0/ 6.
•Resection Margins: Free deep margin.
•Histologic response to therapy: Complete pathological response.
Other pathological findings: Marked calcification. Fibrosis and adenosis in surrounding breast tissue.

Diagnosis:
Left breast mass, Left simple mastectomy specimen & sentinel lymph nodes:
•No remanats of tumour tissue.
•Complete pathological response.
•Free Nipple and areola.

Verified by :
Dr.Marwa mahmoud

Patient Name	: الأستاذة / ثريا يحي محمد علي عطوه		
Patient No.	: 29003140101701	Request No.	: 13000185
Path No.	: 1162/2022	Request Date	: 27/04/2022
Sex	: Female	Verification Date	: 16/05/2022
Age	: 32 Y	Financial Category	: اورام دار السلام - فئة الدولة
Referreing Doc :		Department	: Out Patient
		Registered by	: Ali mosta fa

Pathology Report

- Free Resection margins.
- Free sentinel lymph node 0/ 6.
- Stage: ypT0, ypN0.

Prof.Dr. Gamal Abdellaty,
Dr.Esraa Elgendy.

Verified by :
Dr.Marwa mahmoud

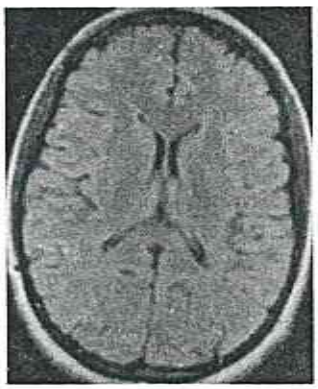
[[DOCTYPE:MRI]]
M.R.I

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : shimaa yahia moahmed ali	**Date : 17-02-2026 01:41**
Branch : Portable	**Medical No. : GRI-260217134123STD131221107523748902**
Ref.Physician Prof : MRI BRAIN+C	**Ins.Company :**

MR EXAMINATION OF THE BRAIN

TECHNIQUE:



Axial T1, T2, FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs
Post contrast T1 WIs

FINDINGS:

- Normal size and configuration of the ventricular system.
- No midline shift or deformity.
- No evidence of cerebral, cerebellar or brain stem areas of abnormal signal.
- Normal cortical sulci and extra axial CSF spaces.
- No evidence of extra axial collections.
- No cerebello pontine angle masses.
- No evidence of posterior fossa abnormalities.
- Intact cervico medullary junction.
- Patent signal void arteries of the circle of Willis and major dural sinuses.

OPINION:

Normal MRI of the brain.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

[[DOCTYPE:CT_SCAN]]

جوستاف روسي

Patient's Name:	**shaimaa ed ali**
Date:	**2026-04-..**
Referred by:	Prof.Dr. ct cap iv
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent left MRM. No available previous films for comparison.

I- CHEST:

- Left upper lung lobe fine subpleural reticulations likely post radiation sequelae.'
- Left upper lung lobe tiny subpleural pulmonary nodule measuring 5 mm in diameter.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Mesenteric panniculitis.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Few subcentimetric bilateral external iliac lymph nodes.
- No ascites.

Opinion:

❖ **iny left upper lobe pulmonary nodule ? inflammatory for follow up.**

Dr.sherif Wagdy

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Patient's Name:	SHAIMAA YAHIA MOHAMMED
Date:	2026-04-11
Referred by:	Prof.Dr.
Hospital:	Goustave Roussey International Egypt

RIGHT MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: *Follow up after left mastectomy.*

- **Breast density: Scattered densities of glnadular tissue** ... ***Matching ACR B.***
- No detected suspicious mass density or architectural distortion.
- No detected pathological micro-calcification.
- Normal skin and nipple.

By breast ultrasound revealed:

- Heterogeneous (predominantly glandular) parenchyma of the right breast.
- Stationary right 12 o'clock benign looking lesion ,it measures (9 x 4 mm).
- *No detected suspicious solid or cystic masses.*
- No dilated ducts.
- Normal skin and nipple.
- Non specfic axillary lymph nodes.
- *NB: Complementary US done at the operative bed. Claer operative bed .No masses .*

CONCLUSION:

- ***Right breast : UOQ benign lesion .BI-RADS 2category.***

Prof.MONA ZAKY

[[DOCTYPE:MRI]]
M.R.I

Patient's Name:	**Shaima Yahya Mohammed Ali Atieh**
Date:	**2026-08-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MRI EXAMINATION OF THE CERVICAL SPINE

Technique of examination:

- **Sagittal T1 & T2 WIs "Plate 1 & 2".**
- **Axial GR/T2 WIs "Plate 3".**
- **No contrast was given.**

MRI Findings:

- Straightened cervical curvature.
- Normal diameters of the bony cervical spinal canal.
- No evidence of disc herniation or significant bulges.
- Normal MRI appearance of the cervical spinal cord and the cranio-cervical junction with no evidence of cord malacia, edema or syrinx formation.
- Normal marrow signal of the imaged vertebrae.
- No paraspinal signal abnormalities.

Conclusion:

- **Neck muscle spasm.**
- **No neurally significant disc lesions.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

[[DOCTYPE:CT_SCAN]]
CT Scan

جوستاف روسي
انترناشونال

Patient's Name:	**shaimaa ıed ali**
Date:	**2026-04-11**
Referred by:	Prof.Dr. **ct cap iv**
Hospital:	**Goustave Roussey International Egypt**

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